ROTATION: NURSERY DAYCARE

Chapter 5 — Nursery Rotation

Daily care and health supervision of healthy children 0-3 years in a daycare/nursery setting: intake assessment, morning health checks, feeding and hygiene procedures, growth monitoring with developmental observation, common problems, and medication/emergency readiness.

Age range: Birth – 3 years

5.1 Site Profile — Role of the Nursery

Many mothers work outside the home, there are many single-parent families, and many families live far from relatives — in all of these circumstances infants may need to be cared for outside the home, in child care settings or home day care centers. The nursery rotation is where the nurse observes and supports that daily care.

- **Types:** sitter in the home, traditional (freestanding) day care center, or home-based day care center with fewer children.
- **Licensing:** families using a freestanding or home-based center should make sure the provider is appropriately **licensed**.
- **Caregiver-to-child ratio:** parents should feel comfortable with the ratio; the number of caregivers per child follows the licensed facility's staffing plan.
- **Staff training:** caregivers should be trained in **infant CPR and first aid** — and parents should be taught first-aid measures and infant/child CPR as well, to minimize the consequences of injury.
- **Separation anxiety:** it is helpful to visit the center once or twice beforehand so the infant gets used to the caregivers from the comfort of the parent's lap. Warn parents that separation anxiety in late infancy can cause a disturbing crying episode when the parent leaves — and reassure them the infant will not suffer harm from the separation.

NURSE'S ROLE IN THE NURSERY

- **Daily care skills** the infant needs: feeding, bathing, and changing diapers.
- **Health surveillance:** morning health checks, growth monitoring, and observation of development — day care providers also play a role in support and education of the family.
- **Teaching:** anticipatory guidance on feeding, safe sleep, safety in the water, home safety, and when to call the provider (see Sections 5.3-5.6).
- Facility licensing, exclusion, and medication policies follow each center's written rules; this chapter covers the clinical content shared by all centers.

5.2 Intake Health Assessment

Admission to the nursery starts with a health history and immunization verification, a baseline physical assessment, and a written emergency plan. The same periodic-screening principle applies in the daycare as in the clinic: periodic screening for adequate growth and development, adherence to the recommended screenings, and the current immunization schedule.

Intake item	What to record
Identification & contacts	Name, date of birth, age, parent(s)/guardian(s), alternative emergency contacts; who may pick the child up
Health history	Interval history — feeding, elimination, sleep, behavior, past illnesses, allergies, medications, and the immunization record
Immunization verification	Review the immunization record against the current schedule; note live-vaccine timing (see Ch. 4 §4.5) and catch-up needs
Birth history & newborn screens	Gestational age, birth weight, newborn screen results, hearing screen (OAE) and CCHD screen results, vitamin K and HepB at birth (Ch. 3 §3.5)
Baseline physical	Weight, length, head circumference, vital signs; skin (rashes), eyes/ears, mouth (teeth present), abdomen, and developmental stage noted (Ch. 3 newborn exam; Ch. 4 infant/toddler exam)
Special needs	Diet (breast/formula/solids), allergies (food, drug, latex), medications to be given during the day, sleeping habits/routine

Emergency plan	Written action plan for injury, choking, allergic reaction, and fever; staff trained in infant CPR and first aid; documentation route (Ch. 5.7, Ch. 9)

FEEDING ACCOUNTABILITY

If breast milk or formula is being expressed/stored at the center, apply the handling rules from Ch. 3 (breast-milk: never microwave; bottle never propped; formula prepared by exact package instructions).

5.3 Daily Health Checks

A quick morning check before the child is accepted tells the nurse whether the child is well enough to be in the group. It observes what can be seen without invasive testing: appearance, behavior, and the first signs of illness.

Check	Normal focus	Prompt a call home / provider (red-flag examples)
Appearance & behavior	Alert, interactive, plays and feeds normally for age	Lethargic, unresponsive, cries constantly, refusal to feed, "not acting like himself/herself"
Temperature	Axillary temperature in the normal range (Ch. 1, Ch. 3: axillary 36.5-37.6°C newborn)	Fever by axilla; any fever in the first months — see facility policy and provider
Skin / rash	No new rash; clean skin	New widespread rash, spreading spots, or rash with fever — evaluate (rule of thumb below)
Feeding / hydration	Feeds as usual; wet diapers count consistent with intake	Poor feeding, forceful vomiting, diarrhea — evaluate; infants need about six wet diapers per 24 hours as an intake indicator
Sleep / alertness	Rested, normal appetite	Sleepy all day, feverish, won't wake for feeding

RULE OF THUMB FOR RASHES AND FEVER

A fever (temperature above the normal range) together with a new generalised rash, lethargy, or poor feeding is a **call-the-provider** situation. Teething does not cause high fever, vomiting, or diarrhea — a temperature elevation with those signs means illness, not teeth. Exclusion criteria (for example "keep home until fever-free for 24 hours") follow each licensed facility's written policy.

5.4 Daily Care Procedures

Feeding by stage (0-3 years)

- **Infant fluid & calories:** fluid 140-160 mL/kg/day (newborn); ~100 mL/kg/day for the first 10 kg plus 50 mL/kg/day for the next 10 kg; calories 105-108 kcal/kg/day (1-6 months: 108 kcal/kg; 6-12 months: 98 kcal/kg).
- **Breastfeeding at the center:** expressed breast milk fed by bottle — never microwaved; feed on the baby's cues; the properly latched infant sucks rhythmically with audible swallowing.
- **Formula:** prepare exactly per package instructions; 2-3 oz per feeding about 6×/day as a starter guide; never prop the bottle and never put juice or sweetened liquid in a bottle.
- **Solids:** begin with a single-ingredient iron-fortified cereal, then fruits and vegetables; introduce one new food at a time and watch for reactions (Ch. 4 §4.4 infant).
- **Toddler meals:** 3 meals + 2 snacks, small portions; milk 2-3 servings (24-28 oz/day equivalent); juice capped at 4-6 oz/day of 100% juice in a cup.
- **Choking prevention while feeding:** slowly add more difficult-to-chew foods as the toddler becomes adept; cut all foods into bite-size pieces; avoid nuts, gumdrops/chewy candies, raw carrots, peanut butter alone, and popcorn; cut hot dogs and grapes into quarters; cook carrots until soft or grate them; **always supervise the toddler while eating**.
- **Self-feeding:** child-sized spoon and fork with dull tines; seat the toddler securely with feet supported, not dangling; never leave the toddler unattended while eating; minimize distractions (turn off the TV).

Hygiene, bathing & diapering

- **Bathing safety:** infants can drown in a very small amount of water — never leave an infant unattended in a sink, baby bathtub, standard bathtub, swimming or wading pool, or any body of water even if shallow; keep the bathroom door closed and the toilet lid down; empty tubs, pails, and buckets immediately after use.

- **Diapering:** changing diapers is part of the infant's daily care; the wet-diaper count (about six per 24 hours) is the caregiver's daily hydration check. Follow the center's universal-hygiene procedure at every change (gloves, cleaning, hand hygiene, safe disposal).
- **Hand hygiene:** handwashing is part of infant-care practice; apply standard infection-control hand hygiene before and after every child contact, meal, and diaper change.

Safe sleep & teething

- **Safe sleep:** place infants on the back for sleep (SIDS risk reduction); cribs must not have pillows, comforters, stuffed animals, or other soft items; keep plastic bags of any size away from infants; keep window-blind and drapery cords out of the infant's reach.
- **Teething:** first lower central incisors appear ~6-10 months (average 8 months); cold items are soothing; fever, vomiting, or diarrhea are *not* teething signs — treat them as illness.

Play & stimulation

- **Infants:** toys recommended for 0-12 months only; avoid stuffed animals with eyes or buttons that can be dislodged; keep the floor free of small dropped items.
- **Toddlers:** parallel play (alongside, not with, others) with imitation — the best "toy" is adult-child interaction.
- Reading, storytelling, and interactive adult-child communication positively affect language development.

5.5 Growth Monitoring & Developmental Observation

Growth and development are monitored at every contact. The nurse weighs and measures on a routine schedule, plots on the correct chart, and observes milestones during daily care.

Element	Practice
Length	Recumbent length on a **length board** until 24 months (or 36 months with the birth-to-36 chart); tapes, wall charts, and flip-up bars are not used
Weight	Nude (or consistent clothing), balance-beam scale (Ch. 1); record the time because height/length is less in the afternoon than in the morning
Head circumference	Measured at every visit in the first years (Ch. 4 Fig. 4.1)

Charts	WHO growth standards 0-2 years; CDC charts beyond 2 years (Ch. 1, Ch. 4 §4.2)
Plotting rule	Compare against the child's own previous curve — a single low percentile is not abnormal by itself (Ch. 4 §4.2)
Milestone observation	Noted during routine care: smile, head control, sitting, pulling to stand, crawling, first words, tantrums, toilet-training markers (see Ch. 4 §4.4 and §4.6)
Red flags	No smile at people by 3 months; no head support by 3-4 months; no sitting with assistance at 6 months; no creeping/crawling and no interest in peek-a-boo at 8 months; not pulling to stand by 11-12 months; no single words with meaning at 12 months

Figure 5.1 charts the morning-check decision: a child with a normal appearance, behavior, temperature, skin, and appetite is accepted into the group, while red flags in appearance, temperature, skin, feeding, or alertness send the child to evaluation and a call home. Exclusion specifics follow the licensed center's written policy.

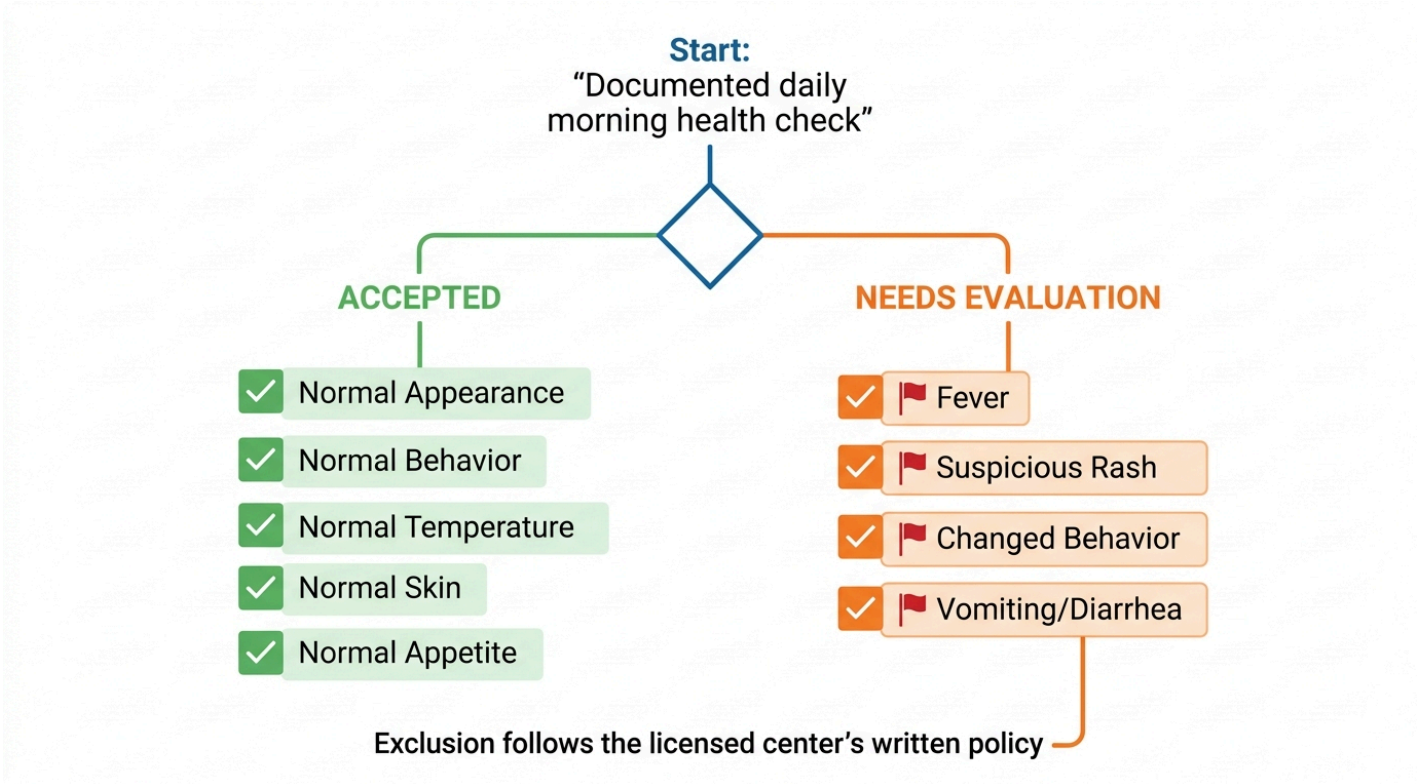


Figure 5.1 Nursery acceptance decision flow after the documented morning check.

5.6 Common Problems in the Nursery

Problem	Guidance
Infections	Infants/children commonly develop infections when entering day care or preschool because of new exposure; apply routine hygiene and exclusion policy; hand hygiene at every contact
Dehydration risk	Poor feeding, forceful vomiting, or diarrhea warrant evaluation; monitor intake/output (wet diapers count)
Diaper-area care	Clean and dry with each change (daily-care skill); creams per pediatrician advice
Choking	Infants bring small items to the mouth — only 0-12-month toys, no dislodgable eyes/buttons, floor kept clear; toddlers have the food-avoidance list above and must be supervised while eating
SIDS prevention	Back-to-sleep; empty crib (no pillows/comforters/stuffed animals); cords out of reach
Poisoning prevention	Medications, household cleaning supplies, and hazardous substances stored completely out of reach; cabinets and drawers secured with child safety latches; approved safety covers on electrical outlets; all medications have child-safety caps and none within the toddler's reach
Drowning risk	Never leave an infant unattended in any water; bathroom door closed, toilet lid down, tubs/pails/buckets emptied; the highest drowning rate occurs in 1- to 2-year-olds — constant supervision in and around all water

5.7 Medication & Emergencies

SAFE MEDICATION HANDLING IN THE NURSERY

- All medications have **child-safety caps**; never leave any medication within the toddler's reach.
- Medications, household cleaning supplies, and other hazardous substances are stored completely out of reach.
- Be especially careful with medications provided in child-resistant containers — "child-resistant" is not "child-proof".
- Administration, dose check, documentation, and parent permission follow the licensed center's written policy.

- **First-aid & CPR readiness:** providers/caregivers should be trained in infant CPR and first aid, and parents should be taught first-aid measures and infant/child CPR to minimize the consequences of injury.
- **Emergency action plan:** a written plan covering choking, injury, allergic reaction, and fever; staff roles, call sequence, who documents, and parent notification. Call-for-help numbers follow the facility's emergency plan.
- **Documentation:** incident reports, medication records, and daily observation logs (report templates in Ch. 9).

Chapter 5 Learning Check

Self-assessment questions (answer in your notebook, then open)

End of Chapter 5.